

Someone texts you:

“Okay.”

That's it.

One word.

Four letters.

A full stop that somehow feels like a personal attack.

You stare at it.

Okay?

You read it again.

Okay.

You start thinking about the conversation before that.

Was I being annoying?

You scroll up.

You reread what you sent.

It looked normal when you sent it.

Now it looks suspicious.

You start analysing punctuation.

Maybe they would have said “okayyy” if everything was fine.

Maybe the full stop means they're irritated.

Maybe they are tired of you.

Maybe you've been talking too much lately.

Maybe they complained about you to someone else.

Maybe you should apologise.

But apologise for what?

You don't know.

So now you're sitting there, emotionally preparing for the end of a relationship that may not even be in trouble.

And the other person?

They probably put their phone down and went to make tea.

This is the strange thing about our brains. Sometimes a tiny event can become enormous inside our heads.

A delayed reply.

A change in someone's tone.

A friend cancelling plans.

A boss saying, "Can we talk?"

Someone watching your message and not replying.

Someone replying with fewer words than usual.

Someone forgetting to invite you somewhere.

Someone saying, "I didn't mean it like that."

And suddenly you are investigating the emotional equivalent of a murder scene.

For some people, this kind of reaction has been given a name: **rejection sensitive dysphoria**, or RSD.

I find the name interesting.

I also think the way we talk about it online deserves a little more caution.

Because this is where mental health gets complicated.

We have more language than previous generations had. We have access to information our parents and grandparents could never have imagined. We can type three symptoms into Google and find twenty explanations before breakfast.

That can be incredibly helpful.

It can also send us down a rabbit hole.

The relief of finally finding a word

There is something almost emotional about discovering a mental health concept that describes something you've struggled to explain.

You read a description and think:

Wait. Other people do this too?

That moment can feel like relief.

Maybe you're not dramatic.

Maybe you're not needy.

Maybe you're not deliberately difficult.

Maybe there is a reason criticism hurts you so much.

Maybe there is a reason you replay conversations.

Maybe there is a reason rejection feels less like a small social inconvenience and more like someone has reached inside your chest and squeezed something.

This is partly why mental health language has become so powerful.

A name can give shape to an experience.

Before you know the word anxiety, you might think you're simply bad at handling things.

Before you know about OCD, you might think you're weird because your brain won't stop producing thoughts you don't want.

Before you understand ADHD, you might spend years believing you're lazy because you can't seem to organise yourself in the way everyone else apparently can.

And before you hear the phrase rejection sensitivity, you might think:

Why am I like this?

That question can be exhausting.

Sometimes finding the language is the first time you stop blaming your personality for something you don't fully understand.

Then came TikTok

Of course, there is another side to this.

We now live in the age of the thirty second diagnosis.

“Five signs you have ADHD.”

“Things people with anxiety do without realising.”

“Three signs you have childhood trauma.”

“POV: you have rejection sensitive dysphoria.”

And you're watching the video thinking:

That's me.

Next video.

That's also me.

Next one.

Oh my God.

Suddenly you've diagnosed yourself with ADHD, OCD, anxiety, autism, attachment issues, childhood trauma and possibly a personality disorder before you've even finished your coffee.



The internet can be ridiculous.

Human beings are complicated.

Mental health conditions are complicated.

A list of seven relatable behaviours cannot tell you everything about what is happening inside your mind.

Everyone procrastinates sometimes.

Everyone overthinks sometimes.

Everyone forgets things.

Everyone gets nervous.

Everyone checks something twice.

Everyone gets hurt when someone they care about seems distant.

Everyone has intrusive thoughts.

Everyone has bad days.

That doesn't mean everyone has a disorder.

And yet, I don't think the answer is to laugh at people who try to understand themselves through these online resources.

Because sometimes the person watching that video really does have something going on.

Sometimes the person who has spent years thinking they are lazy really does have ADHD.

Sometimes the person who thinks they're "too sensitive" really is experiencing emotions in a way that is unusually intense.

Sometimes the person who has spent years being told to "stop thinking so much" has an anxiety disorder.

Sometimes the thing you've been calling a personality flaw actually has a name.

That's where I think we need some balance.

Self diagnosis isn't automatically nonsense

I'm not a fan of turning every behaviour into a diagnosis.

I also don't think self diagnosis deserves the automatic eye roll it sometimes gets.

There are people who don't have access to mental health professionals.

There are people who can't afford assessments.

There are people living in places where mental health services are limited.

There are people whose families would never support them seeking help.

There are people who have spent years being dismissed.

So sometimes the internet is where they first encounter the information that makes their life make sense.

That's not nothing.

At the same time, recognising yourself in a list isn't the same thing as receiving a diagnosis.

There is a difference between:

“This sounds familiar.”

and

“I definitely have this condition.”

That little gap matters.

A diagnosis isn't supposed to be a personality quiz.

It's supposed to involve looking at patterns, severity, duration, context, functioning and alternative explanations.

And honestly, sometimes the answer isn't even particularly exciting.

You might not have a disorder.

You might be tired.

You might be grieving.

You might be stressed.

You might be in a terrible relationship.

You might have poor boundaries.

You might be dealing with someone who genuinely treats you badly.

You might simply be having a very human reaction to a very unpleasant situation.

Mental health doesn't have to be pathological for it to matter.

And sometimes the rejection really happened

This is another part of the conversation I think gets lost.

We talk so much about “perceived rejection” that we can accidentally make people question their own experiences.

Sometimes you are not imagining the change in someone's behaviour.

Sometimes they really are annoyed.

Sometimes someone really is pulling away.

Sometimes a friend really has become distant.

Sometimes someone really did insult you.

Sometimes the relationship really is unhealthy.

Not every feeling of rejection is RSD.

Sometimes rejection is simply rejection.

The important question is what happens next.

If someone doesn't reply for six hours and your brain immediately decides they hate you, that might be worth examining.

If someone repeatedly ignores you, excludes you, insults you and tells you you're imagining it, noticing that pattern isn't necessarily hypersensitivity.

We need to be careful about turning every uncomfortable intuition into either “my anxiety is lying to me” or “my intuition is always right.”

Our brains can misinterpret things.

People can also genuinely hurt us.

Both things can be true.

Why does one little thing hurt so much?

This is where RSD becomes particularly interesting.

The term is commonly associated with ADHD, although RSD itself isn't an official psychiatric diagnosis.

People use it to describe an unusually intense reaction to rejection, criticism, failure or disapproval.

And when you hear people describe it, the experience can sound surprisingly physical.

It's not simply:

"Oh, that hurt my feelings."

It can be:

I need to get away from everyone.

I can't stop thinking about this.

I feel humiliated.

I want to disappear.

I'm furious.

I need to fix this immediately.

I never want to speak to that person again.

And then, sometimes, the emotional intensity disappears almost as quickly as it arrived.

That can be confusing.

You can know intellectually that someone probably didn't mean anything by their message while simultaneously feeling like your entire relationship is collapsing.

That's one of the strangest parts of emotional experiences.

Knowing something logically doesn't always make your nervous system believe it.

You can tell yourself:

“They are probably busy.”

And your brain responds:

“Interesting theory. Here are 47 reasons they're actually leaving you.”

Thanks, brain.

Very helpful.

We have always had these experiences

I sometimes wonder what previous generations called these things.

Because people didn't suddenly start having intrusive thoughts when psychologists gave them names.

People didn't suddenly develop attention difficulties when ADHD became widely discussed.

People didn't suddenly become anxious when anxiety disorders entered everyday conversation.

The experiences existed before the vocabulary did.

A person could have spent their entire childhood unable to concentrate, constantly losing things, struggling with organisation and being told they were careless.

Another person could have spent years washing their hands until their skin hurt because they were terrified of contamination.

Another could have spent decades worrying that every small mistake meant everyone secretly disliked them.

They didn't necessarily have the language to describe what was happening.

Maybe they called themselves difficult.

Maybe they called themselves weak.

Maybe they simply thought:

This is who I am.

And that is one of the things I appreciate about modern conversations around mental health.

We have words now.

Words can help.

But our parents weren't completely wrong either

This is where I probably disagree with both extremes.

The older generation sometimes gets painted as people who didn't believe in mental health.

I don't think that's entirely fair.

Many grew up in circumstances where survival was the priority.

You didn't necessarily get to sit down and analyse your childhood.

You had work.

You had children.

You had bills.

You had responsibilities.

You were expected to get up the next morning regardless of how awful you felt.

Mental health care was also less accessible and less openly discussed.

So when someone says:

“We went through worse and survived.”

I understand where that comes from.

But surviving something doesn't automatically mean it didn't hurt you.

And functioning doesn't automatically mean you were fine.

At the same time, I understand the frustration when every ordinary inconvenience is given a clinical label.

There is a difference between having a difficult day and having depression.

There is a difference between liking things organised and having OCD.

There is a difference between being forgetful and having ADHD.

There is a difference between feeling rejected and experiencing a pattern of overwhelming rejection sensitivity.

We don't help anyone by pretending the distinctions don't exist.

Maybe the point isn't to diagnose ourselves

Maybe the better use of all this information is curiosity.

Instead of:

“I saw a video. I have RSD.”

Maybe:

“I saw this description and recognised myself. Why do I react this way?”

That's a much more useful question.

When did I start doing this?

Does it happen with everyone or only certain people?

Does criticism always feel unbearable?

Do I regularly assume people are angry with me?

Do I spend hours replaying conversations?

Does this affect my friendships, work or relationships?

Am I reacting to what actually happened or to what I think might have happened?

Have I always been like this?

Could there be another explanation?

Those questions can take you much further than a checklist.

And if the pattern is severe enough, persistent enough or disruptive enough, maybe that's when you seek professional help.

Not because you failed at understanding yourself.

Because sometimes understanding yourself is worth getting help with.

So about that “Okay.”

Maybe they were annoyed.

Maybe they weren't.

Maybe they were busy.

Maybe they were distracted.

Maybe their phone was at two percent.

Maybe they genuinely didn't know what else to say.

Maybe your brain filled in the silence with the worst possible explanation because uncertainty is uncomfortable.

The message itself was small.

The story your mind built around it was enormous.

And I think that's where the conversation about rejection sensitivity becomes useful.

Not because every painful reaction needs a diagnosis.

Not because every intense emotion means something is clinically wrong with you.

But because sometimes our reactions are telling us something.

Maybe about our history.

Maybe about our relationships.

Maybe about our nervous system.

Maybe about something we have never properly dealt with.

And sometimes, yes, maybe there is a name for it.

The trick is learning the name without allowing the name to become your entire identity.

Because you are allowed to say:

“This sounds like me.”

You are allowed to investigate.

You are allowed to seek help.

You are allowed to discover that your original assumption was wrong.

And you are allowed to discover that you were right.

Mental health isn't about finding the perfect label for every uncomfortable feeling.

Sometimes it's about finally becoming curious about the person experiencing them.

Even when that person is sitting at 1:17 a.m., rereading a four letter text message like it contains classified government intelligence.

And honestly?

We've all been there.